Zoledronate infusion

Zoledronate infusions are given to treat osteoporosis and Paget's disease. They can also prevent some forms of cancer from spreading in the bones. Zoledronate, also called zoledronic acid, is a powerful type of bisphosphonate.

How bisphosphonates work

Your bones constantly renew themselves. Your body breaks down older bone and removes (resorbs) it. Then new bone forms.

As you age, your bone density tends to decrease because your body breaks down bone faster than new bone forms. Bisphosphonates slow the rate that bone is broken down in your body. This maintains bone density and reduces your risk of breaking a bone.

Treatment with zoledronate

Zoledronate is given directly into your arm through a drip. This is called intravenous infusion. It takes between 15 and 30 minutes.

For osteoporosis, a zolendronate infusion is usually given every 18 to 24 months. After a few years your doctor will review your treatment to check if you still need to have infusions.

While the zoledronate medication is free, you may need to pay a prescription fee and to have the infusion.

Benefits of zoledronate

Zoledronate helps to:

- improve bone density and reduce fracture risk in people with osteoporosis and related conditions such as Paget's disease.

- reduce bone pain in conditions such as Paget's disease and some cancers.

Zoledronate can reduce the risk of spinal fractures by 70% and hips fractures by 35%.

Side effects

Some people have flu-like symptoms after their first treatment of zoledronate. Otherwise, side effects are not common.

When considering zoledronate treatment, it is important to balance the small risk of side effects against the much greater risk of a major broken bone.

Flu-like symptoms

About 30% of people have a flu-like symptoms such as muscle and joint pains, fever and headache after their first treatment.

These symptoms usually go away after a couple of days and can be treated with paracetamol. The chances of this side effect happening after the second or third zoledronate infusion is much lower (about 3 to 4%).

Eye inflammation

Less than 10% of people may develop eye inflammation. If you develop symptoms such as a painful red eye or blurred vision contact your healthcare provider.

Rare health risks

Osteonecrosis of the jaw

In extremely rare cases, Zoledronate infusions can damage the healthy bone tissue in the jaw (osteonecrosis).

This happens when healing inside the mouth is delayed, usually after major dental treatment such as extractions or implants. For this reason, your healthcare provider may suggest you delay the infusion until after you have had any major dental treatment you might need.

It is important to maintain good mouth hygiene and have regular dental check-ups.

Upper leg fractures

There is a very small risk of unusual leg fractures after many years of treatment with zoledronate infusions.

Preparing for the infusion

Your own healthcare provider may do your zoledronate infusion or it may be done by another healthcare provider or at a hospital clinic. The following steps need to be done before you have the infusion.

Health check and consent

Your healthcare provider will check that you are able to have zoledronate. They will explain about the medication and what to expect, including possible side effects. Either your healthcare provider or the person performing the infusion will make sure that you are happy to proceed and will ask you to sign a consent form.

Prescription

Your healthcare provider will give you or your pharmacy a prescription for the infusion. You will need to:

- take the prescription to a pharmacy (if you have a paper prescription)
- pick up the infusion before your appointment
- remember to take it with you on the day of your procedure.

Dental check

If you need any major dental treatment, you should have it done before your infusion. If you have not had a recent dental check, consider having one and talking to your dentist about the infusion.

Blood test

Your healthcare provider will arrange for you to have a blood test 1 to 2 weeks before the infusion. This is to check your kidneys are working well and to check the calcium levels in your blood

Vitamin D

If you are not already taking vitamin D tablets, your healthcare provider will prescribe them and tell you when to take them.

On the day of the infusion

On the day of your infusion appointment, you should **not** take any:

- anti-inflammatory medication (like aspirin or ibuprofen)
- diuretic medication (like furosemide or bendroflumethiazide).

If you are not sure whether this applies to any medicines you are taking, ask your healthcare provider.

On the day of your infusion:

- eat and drink as normal
- drink 1 to 2 cups glasses of water before and after the infusion
- take 2 paracetamol tablets 30 to 60 minutes before your appointment.

If you develop flu-like symptoms after your infusion, paracetamol should help. Your healthcare provider may provide this or you can buy it from a supermarket or pharmacy. Drinking plenty of water should also help.

Remember to take the infusion with you to your appointment.

Talk to your healthcare provider before attending your appointment if you have any questions about the procedure, the medicine or your bone condition.

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